



Document Management System



Sultanate of Oman
Ministry of Health
The Royal Hospital
Department of Pharmacy

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Title: Dangerous Abbreviation

1. Purpose

To provide guidance on clinical documentation to facilitate patient safety by ensuring that clinical documentations are clear and free from ambiguous and frequently misinterpreted abbreviations, symbols and dose designations that may potentially lead to harmful medication errors.

2. Policy Statement

Health care professionals at the Royal Hospital will **not use any** of the prohibited abbreviations, symbols and dose designations based on the ISMP Canada recommendations. (Please refer to the Appendix). This list shall be updated based on recommendations from ISMP Canada to reflect evidence-based patient safety.

The abbreviations that are found in the prohibited abbreviation column attached in the Appendix may frequently be misinterpreted and lead to harmful and/ or fatal medications errors. They should never be used when communicating clinical information. In additional to the list of prohibited abbreviation the following is suggested:

- The use of symbols are discouraged.
 - Drug names should be spelled out in full using the generic names. (some cases the brand name may need to be included too.)
 - It is preferred that the Metric system be used for writing dosages.
 - Always use a space between drug name, dose, and unit of measurement.
 - “Left” and “Right” are to be spelled out on all consent forms, and intra-operative/ procedure records.
- Compliance shall be measured through a single point prevalence audit. If full compliance is not achieved, it is the responsibility of the various directors to take steps to address the deficiencies.

3. Definitions

- Abbreviation: A shortened form of a word or phrase.
- Symbol: Something that stands for, or represents, another thing; a written or printed mark, letter, abbreviation standing for an object, quality, process, quantity, etc.

and pharmacy staff who order medications or provide clinical documentation while caring for and treating patients.

2. **Responsibility**

Royal Hospital managers are responsible for ensuring and demonstrating their commitment to patient safety by ensuring that the medication orders (manual prescriptions, pre-printed labels, electronic prescriptions, progress notes, and referral notes) are clear, including being free of all prohibited abbreviations, symbols and dose designations.

In the case of medication orders, if a dangerous abbreviation, acronym, or symbol is used by a prescriber, the order will not be processed from the pharmacy unless the abbreviation, acronym, or symbol is clarified with the prescriber or their designee. Clarification of the order by the prescriber or designee must take place prior to documenting, dispensing, administering, or carrying out the order. Whenever possible the prescriber will rewrite the order clarification. In situations where the prescriber is not physically present to rewrite the order, the clarification may be taken as a telephone order and documented as such.

3. **Principles**

To improve patient safety through effective communication among health care professionals at the Royal Hospital by targeting communication and education strategies to minimize medication errors from the use of ambiguous abbreviations, symbols and dose designations as defined by institute of safe medication practices (ISMP).

6.1 Prohibited abbreviations, symbols and dose designation will not be present on any pre-printed patient care order forms, care plans, guidelines or standards.

6.2 Prohibited abbreviations, symbols and dose designations will not be present on any pharmacy communication forms, medication labels, medication administration record, drug protocols, or drug formulary.

6.3 Prohibited abbreviation, symbols and dose designation will not be used in any transcript of clinical documents.

6.4 Prohibited abbreviation, symbols and dose designation will not be used when giving and/or transcribing verbal and telephone medication orders.

The abbreviations that are found in the prohibited abbreviation column attached in the Appendix may frequently be misinterpreted and lead to harmful and/or fatal medications errors. They should never be used when communicating clinical information's.

4. **Policy elements**

The Royal Hospital administrators will support the rights of health care professionals to question medication orders, prescriptions, labels, electronic prescribing modules, progress notes, and referral notes without penalty when in the professional judgment the order and/or notes presents a risk to the safety of the patient.

Royal Hospital administrators shall demonstrate compliance with this policy by:

- a) Implementing the prohibited abbreviations, symbols and dose designation list in the Appendix (Prohibited Abbreviations, Symbols and Dose Designations – DONOT USE).
- b) Providing staff education during orientation programs and when changes are made.
- c) Facilitating annual single point prevalence audit on prohibited abbreviation from medication orders (prescriptions and labels, electronic prescribing modules, progress notes, and referral notes).
- d) Ensuring issues and concerns identified by regular audits for compliance are acted upon.

5. **Communication Strategies**

The “Prohibited Abbreviations, Symbols and Dose Designations – “DO NOTUSE” list will be communicated all to all the healthcare professional at the Royal Hospital.

Methods of communication include, but are not limited to:

- A copy of the ‘Prohibited Abbreviations, Symbols and Dose Designations – DO NOT USE’ will be accessible in the Royal Hospital portal.
- Posters on dangerous abbreviations will be distributed in the wards/units, doctors room, computer

6. Monitoring

The Royal Hospital leaders will randomly audit patient charts, utilizing a point prevalence audit, to determine compliance with this policy. At any time, orders found to be non-compliant with the policy may be reported into the Royal Hospital electronic incident reporting system.

7. Clinical documents

Definitions of clinical documents include, but are not limited to:

- admission history
- discharge summaries
- medication profiles
- medication orders
- handwritten medication orders
- progress notes
- medication administration records
- consultation reports
- operative reports
- pre-printed orders, including diagnostics
- Protocols, guidelines, standards
- drug formularies
- diagnostic result reports
- pharmacy labels

Appendix

Prohibited Abbreviations	Intended Meaning	Misinterpretation	Correction
µg	Microgram	Mistaken as “mg”	Use “mcg”
AD	Alternative day	Mistaken as “right ear”	Use “Alternative day”
BT	Bedtime	Mistaken as “BID” (twice daily)	Use “bedtime”
cc	Cubic centimeters	Mistaken as “U” (units)	Use “mL”
D/C	Discharge or discontinue	Premature discontinuation of medications if D/C (intended to mean “discharge”) has been misinterpreted as “discontinued” when followed by a list of discharge medications	Use “discharge” and “discontinue”

IN	Intranasal	Mistaken as “IM” or “IV”	Use “intranasal” or “NAS”
HS hs	Half- strength At bedtime, hours of sleep	Mistaken as bedtime Mistaken as half-strength	Use “half- strength” or “bedtime”
IU**	International unit	Mistaken as IV (intravenous) or 10 (ten)	Use “units”
o.d or OD	Once daily	Mistaken as “right eye” (OD-oculus dexter), leading to oral liquid medications administered in the eye.	Use “daily”
OJ	Orange juice	Mistaken as OD or OS (right or left eye); drugs meant to be diluted in orange juice may be given in the eye	Use "orange juice"
Per os	By mouth, orally	The “os” can be mistaken as “left eye” (OS-oculus sinister)	Use “PO”, “by mouth
q.d. or QD**	Every day	Mistaken as q.i.d., especially if the period after the “q” or the tail of the “q” is misunderstood as an “i”	Use “daily”
qhs	Nightly at bedtime	Mistaken as “qhr” or every hour	Use “nightly”
Qn	Nightly or at bedtime	Mistaken as “qh” (every hour)	Use “nightly” or “at bedtime”
q.o.d or QOD	Every other day	Mistaken as “q.d.” (daily) or “q.i.d. (four times daily) if the “o” is poorly written	Use “every other day”
q1d	Daily	Mistaken as q.i.d. (four times daily)	Use “daily”

q6PM, etc.	PM	Mistaken as every 6 hours	PM daily”
SC, SQ, sub q	Subcutaneous	SC mistaken as SL (sublingual); SQ mistaken as “5 every;” the “q” in “sub q” has been mistaken as “every” (e.g., a heparin dose ordered “sub q 2 hours before surgery” misunderstood as every 2 hours before surgery)	Use “subcut” or “subcutaneously”
Ss	Sliding scale (insulin) or ½ (apothecary)	Mistaken as “55”	Spell out “sliding scale;” use “one-half” or “½”
SSRI SSI	Sliding scale regular insulin Sliding scale insulin	Mistaken as selective-serotonin reuptake inhibitor Mistaken as Strong Solution of Iodine (Lugol's)	Spell out “sliding scale (insulin)”
i/d	One daily	Mistaken as “tid”	Use “1 daily”
TIW or tiw	3 times a week	Mistaken as “3 times a day” or “twice in a week”	Use “3 times weekly”
U or u	Unit	Mistaken as the number 0 or 4, causing a 10-fold overdose or greater (e.g., 4U seen as “40” or 4u seen as “44”); mistaken as “cc” so dose given in volume instead of units (e.g., 4u seen as 4cc)	Use “unit”
UD	As directed (“ut dictum”)	Mistaken as unit dose (e.g., diltiazem 125 mg IV infusion “UD” misinterpreted as meaning to give the entire infusion as a unit [bolus] dose)	Use “as directed”
Dose Designations and Other Information	Intended Meaning	Misinterpretation	Correction
Trailing zero after decimal point (e.g., 1.0 mg)**	1 mg	Mistaken as 10 mg if the decimal point is not seen	Do not use trailing zeros for doses expressed in whole numbers

Naked decimal point (e.g., .5 mg)**	0.5mg	Mistaken as 5 mg if the decimal point is not seen	point when the dose is less than a whole unit
Abbreviations such as mg. or mL. with a period following the abbreviation	mg mL	The period is unnecessary and could be mistaken as the number 1 if written poorly	Use mg, mL, etc. without a terminal period
Drug name and dose run together (especially problematic for drug names that end in “l” such as Inderal40 mg; Tegretol300 mg)	Inderal 40 mg Tegretol 300 mg	Mistaken as Inderal 140 mg Mistaken as Tegretol 1300 mg	Place adequate space between the drug name, dose, and unit of measure
Numerical dose and unit of measure run together (e.g., 10mg, 100mL)	10 mg 100 mL	The “m” is sometimes mistaken as a zero or two zeros, risking a 10- to 100-fold overdose	Place adequate space between the dose and unit of measure
Large doses without properly placed commas (e.g., 100000 units; 1000000 units)	100,000 units 1,000,000 units	100000 has been mistaken as 10,000 or 1,000,000; 1000000 has been mistaken as 100,000	Use commas for dosing units at or above 1,000, or use words such as 100 "thousand" or 1 "million" to improve readability
Drug Name Abbreviations	Intended Meaning	Misinterpretation	Correction
To avoid confusion, do not abbreviate drug names when communicating medical information. Examples of drug name abbreviations involved in medication errors include:			
APAP	Acetaminophen	Not recognized as acetaminophen	Use complete drug name
AZT	zidovudine (Retrovir)	Mistaken as azathioprine or aztreonam	Use complete drug name
CPZ	Compazine (prochlorperazine)	Mistaken as chlorpromazine	Use complete drug name
HCl	Hydrochloric acid or hydrochloride	Mistaken as potassium chloride (The “H” is misinterpreted as “K”)	Use complete drug name

HCTZ	Hydrochlorothiazide	as HCT250 mg)	Use complete drug name
MgSO4**	magnesium sulfate	Mistaken as morphine sulfate	Use complete drug name
MS, MSO4**	morphine sulfate	Mistaken as magnesium sulfate	Use complete drug name
MTX	Methotrexate	Mistaken as mitoxantrone	Use complete drug name
PTU	propylthiouracil	Mistaken as mercaptopurine	Use complete drug name
ZnSO4	Zinc sulfate	Mistaken as morphine sulfate	Use complete drug name
Stemmed Drug Names	Intended Meaning	Misinterpretation	Correction
“Nitro” drip	nitroglycerin infusion	Mistaken as sodium nitroprusside infusion	Use complete drug name
“IV Vanc”	Intravenous vancomycin	Mistaken as Invanz	Use complete drug name
Symbols	Intended meaning	Misinterpretation	Correction
x3d	For three days	Mistaken as “3doses”	Use “for three days”
/ (slash mark)	Separates two doses or indicates “per”	Mistaken as the number 1 (e.g., “25 units/10 units” misread as “25 units and 110” units)	Use “per” rather than a slash mark to separate doses
@	At	Mistaken as “2”	Use “at”
&	And	Mistaken as “2”	Use “and”
+	Plus or and	Mistaken as “4”	Use “and”

Written By: Wijdan Hashimani
Checked By: Maya Almawali Asma Alhindi
Authorized by: FAISAL AL-HARTHI



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